

MANAGEMENT OF PERIOPERATIVE THEATRE RESOURCES

Introduction to the unit of learning

This unit standard specifies competencies required to Plan and Prepare to Manage Perioperation Theatre resources, Organise Perioperation Theatre resources, Direct Perioperation Theatre resources, Control Perioperation Theatre resources and Document Perioperation Theatre resources.

Summary of Learning Outcomes

- 1) Plan and Prepare to Manage Perioperation Theatre resources
- 2) Organise Perioperation Theatre resources
- 3) Direct Perioperation Theatre resources
- 4) Control Perioperation Theatre resources
- 5) Document Perioperation Theatre resources

Learning Outcome 1: Plan and Prepare to Manage Perioperation Theatre resources

Introduction to the learning outcome

Upon completion of this unit, the trainees should be able to demonstrate knowledge, skills and attitude on basic management, management theories, perioperation theatre resources, Theatre layout and Resource mobilization.

Performance Standard

1. Operation theatre resources are identified according to Standard Management Principles
2. PT resources are gathered according to Standard Management Principles and work place policy
3. Condition of PT resources is verified as per Standard Management Principles

Definition of terms

Theory: a supposition or a system of ideas intended to explain something, especially one based on general principles independent of the thing to be explained

Resources: a stock or supply of money, materials, staff, and other assets that can be drawn on by a person or organization in order to function effectively

Policy: a course of action adopted and pursued by a government, ruler, political party, etc.: our nation's foreign policy. action or procedure conforming to or considered with reference to prudence or expediency: It was good policy to consent

Content/procedures/methods/illustrations

Plan and Prepare to Manage Perioperation Theatre resources

Management: is the coordination and administration of tasks to achieve a goal. Such administration activities include setting the organization's strategy and coordinating the efforts of staff to accomplish these objectives through the application of available resources.

Basic management

What Is Management?

Management is essential to any organization that wishes to be efficient and achieve its aims. Without someone in a position of authority there would be organizational anarchy with no structure and very little, if any focus. It has been said that management has four basic functions – planning, organizing, leading and controlling. Common sense dictates that without these principles of management being in place an organization would have trouble achieving its aims, or even coming up with aims in the first place! A classic theory on the principles of management was written by Henri Fayol. It seeks to divide management into 14 principles. We'll take a look at these basic principles of management and explain them in easy to understand terminology.

Principles of Management – Fayol's 14 Principles

Division of Work – This principle of management is based on the theory that if workers are given a specialized task to do, they will become skillful and more efficient in it than if they had a broader range of tasks. Therefore, a process where everyone has a specialized role will be an efficient one.

Authority – This principle looks at the concept of managerial authority. It looks at how authority is necessary in order to ensure that managerial commands are carried out. If managers did not have authority then they would lack the ability to get work carried out. Managers should use their authority responsibly and ethically.

Discipline – This principle relates to the fact that discipline is needed within an organization for it to run effectively. Organizational rules, philosophies, and structures need to be met. In order to have disciplined workers, managers must build a culture of mutual respect and motivation.

Unity of command – There should be a clear chain of command in place within an organization. An employee should know exactly whose instructions to follow.

Unity of direction – Work should be organized in a way that means employees are working in harmony toward a shared objective or goal using a shared method or procedure.

Subordination individual interests to the collective interests – The interests of the organization as a whole should take precedence over the interests of any individual

employee or group of employees. This encourages a team spirit and collective mentality of all for one and one for all.

Remuneration – In order to motivate and be fair to employees, they should be paid a reasonable rate for the work they carry out. An organization that underpays will struggle to attract quality workers who are motivated.

Centralization – This principle relates to whether decisions should be made centrally, as in from the top down, or in a more democratic way, from the bottom up. Different decision-making processes are appropriate for different types of decisions.

Scalar chain – This relates to the principle of a clear chain of communication existing between employees and superiors. The chain should be respected, unless speedy communication is vital, in which case the chain may be bypassed if all parties consent.

Order – This relates to the proper use of resources and their effective deployment in a structured fashion.

Equity – Managers should behave ethically towards those they manage. Almost every organization in the modern world will have a written set of policies and procedures which will outline exactly what is expected from staff at all levels.

Stability of tenure of personnel – It is seen as desirable within an organization to have a low staff turnover rate. This is due to the benefits that come with having experienced staff and the time and expense needed to train new ones. There should be a clear and efficient method of filling any staff vacancies that arise.

Initiative – Employees that have an input as to how to best do their job are likely to feel more motivated and respected. Many organizations place a great deal of emphasis on listening to the concerns of staff.

Morale – Keeping a high level of morale and team spirit is an essential part of having the most productive organization possible. Happy and motivated employees are far more likely to be productive and less absent.

Management theories

There are two major but opposing schools of thought in management: scientific management and the human relations-based approach. As its

The four main classifications of management:

- Classical Theories (Scientific Management)
- Human Relations or Behavioural Approach Theories • Systems Theories
- Contingency Theories

What is Classical Management Theory?

- Classical management theory involves creating multiple levels of workers to improve productivity.
- Employees at the lowest levels find their tasks overseen by supervisors who, in turn, are overseen by managers. At every level, employees are expected to perform tasks according to specific procedures designed to maximize productivity.
- In addition, this theory focuses on an impersonal side of business. Employees and managers should not allow friendliness and personal interactions to become involved with the organization. Rules must be followed exactly, and the hiring and firing of employees must relate only to the skills they possess

Classical Theories:

Scientific Approach to Management

The approach relied on systematic information collection, analysis and identification of causes and effects, followed by effective organization o management structure.

The objective was to develop basic principles that could guide the design, creation and maintenance of organizations. The emphasis was on efficiency and effectiveness. Some of the prominent figures that may be identified with the classical school of thought are:

1. Frederic W. Taylor
2. Henri Fayol
3. Max Weber

1. Frederic W. Taylor

Frederic W. Taylor, an engineer, was instrumental in conducting research on methods of training workers for increased production. He believed in the principle 'best management is a true science'.

- According to Taylor the objective of management is to secure the maximum prosperity for the employer, as well as each employee. Taylor's system for work improvement consisted of the following steps:
- Observing the workers' performance through time and motion to determine the best way to carry out each task.

- Scientifically selecting the best worker to perform each job, that is, the person with the characteristics and abilities needed to carry out tasks efficiently.
- Training the selected worker to perform tasks in the most efficient manner.
- Appointing a few highly skilled workers in managerial positions.
- Most of the views expressed by Taylor can be, and are, applied in nursing, where our aim is to recruit the best qualified workers, train them and appoint them to precise specialized positions.
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2. Henri Fayol

- Henri Fayol came up with the functions of a manager, which he identified as planning, organizing, coordinating and controlling. He also identified a set of fourteen principles which managers might apply as they perform their duties. Some of these include the division of labour and specialization, chain of command, centralization and responsibility.

Fayol, Five Basic Functions Management

- Planning: Deciding what needs to happen in the future and generating plans for action (deciding in advance).
- Organizing nonhuman
(or staffing): Making sure the human and resources are put into place
- Coordinating: Creating a structure through which an organization's goals can be accomplished.
- Commanding (or leading): Determining what must be done in a situation and getting people to do it.
- Controlling: Checking progress against plans

3. Max Weber

- Max Weber advocated bureaucracy as the ideal form of organization for a complex institution. He described Bureaucracy as having a well defined hierarchy of authority,

pyramidal in shape, division of labour based on specialization and highly specific rules governing employers' duties and rights. He claimed that bureaucracy was superior because it provided stability and reliability in controlling employees.

Although Weber meant well, the term bureaucracy implies slowness and inefficiency in today's society. Bureaucracies are often seen as too rigid to respond to current and rapid changes in the society.

Behavioral Approach /Human Relations Theories

- Abraham Maslow's Theory
- Douglas McGregor: Theory x and Theory y • Vroom's Expectancy Theory
- Fredrick Herzberg's Two Factor Theory

What is Human Relations Theory?

• Human relations theory, also known as behavioral management theory, focuses more on the individuals in a workplace than the rules, procedures and processes. Instead of directives coming directly from management, a human relations theory provides communication between employees and managers, allowing them to interact with one another to help make decisions. Instead of giving workers quotas and requiring certain procedures, workers are exposed to motivational and emotional tactics to get them to increase productivity. The focus of this style is creating fulfilled, productive workers and helping workers invest in a company.

1. Abraham Maslow's Theory

• According to Maslow's theory, every human being has basic needs. People are motivated by the desire to satisfy these needs. Once a need is satisfied it ceases to be a motivator. As one need is satisfied, another appears and takes its place, and the individual is then motivated to satisfy the new need.

The theory also assumes that a person can satisfy one need at a time. Maslow, a psychologist, developed the hierarchy of needs with the physiological needs at the bottom, which the individuals must satisfy before realising higher level needs. Maslow visualised needs as arranged in hierarchical order starting with basic needs, progressing to higher needs depending on the individual.

There are more people at the base of the triangle still trying to satisfy the basic needs and fewer people were on the higher subsequent levels. Only a small percentage of people can reach the top of the hierarchy.

Figure 1: Showing Abraham Maslow's Theory



Link:<https://www.google.com/url?sa=i&url=https%3A%2F%2Fwww.simplypsychology.org%2Fmaslow>

- The various needs can be further defined as follows:
- Physiological needs are those needs that are required for survival. These include food, oxygen and sleep, sex and rest.
- Safety needs refer to the need for job security, shelter and a stable environment.
- Love and belonging indicate a need for affectionate relations with others as well as the need to belong to a group.
- Self esteem refers to the need for self respect and recognition. At this stage people strive for power, achievement and status.
- Self actualization is a need for self fulfilment and is the culmination of all other needs.

2. Douglas McGregor: Theory X and Theory Y

- McGregor's theory 'X' and 'Y' are a set of management assumptions about the behavior of subordinates. He noted that most managers make these assumptions about their employees.
- The first sets of assumption are known as Theory 'X' they include:
 - An average human being has an inherent dislike for work and will avoid it if possible.
 - As a result of the assumption above, most people must be coerced, controlled, directed and threatened with punishment in order to produce.
 - The average human being has to be closely directed, wishes to avoid responsibility and only wants security.
- The second set of assumptions known as Theory 'Y' regard people in a more favorable way. They state that:
 - Employees like work, which is as natural as rest or play.
 - Human beings do not have to be controlled or coerced as long as commitment to the organisation is present.
 - Under proper conditions, they will not only accept but also seek responsibility.

3. Vroom's Expectancy Theory of Motivation

- By Victor Vroom.
- It examines motivation from the perspective of why people choose to follow a particular course of action.

Vroom introduces three variables: Valence, Expectancy & Instrumentality

i). Valence ~positivity

- This is the importance that the individual places upon the expected outcome of a situation.

ii). Expectancy~ effort

This is the belief that output from the individual and the success of the situation are linked, for example, if I work harder then the outcome will be better

iii). Instrumentality~ reward

- This is the belief that the success of the situation is linked to the expected outcome of the situation, for example, it's gone really well, so I'd expect praise.

- At first glance, this theory would seem most applicable to a traditional attitude work situation where the level of motivation depends on whether the employee wants the reward on offer for doing a good job and whether they believe more effort will lead to that reward.

- However, it could equally apply to any situation where someone does something because they expect a certain outcome. For example, I recycle paper because I think it is important to conserve resources and take a stand on environmental issues (valence), I think that the more effort I put into recycling, the more paper I will recycle (expectancy) and I think that the more paper I recycle, the less resources will be used (instrumentality)

4. Fredrick Herzberg's Two Factor Theory (Dual Structure Theory)

- Frederick Herzberg proposed a theory of motivation based on the idea that some factors motivate and some demotivate. He called this theory the 'Two Factor Theory'. According to Frederick Herzberg, these factors fall into two categories.

I. Motivation Factors ii. Hygiene Factors

- what motivates workers and what demotivates them are two separate elements that act independently of one another.

- Motivators, such as challenging work, recognition and responsibility, will lead to increased satisfaction and commitment to the organization.

- Hygiene factors, such as salary, job security, supervisory practices and benefits, do not actually motivate employees, but will demotivate them if they are poor or missing.

Hygiene Factors

- Hygiene factors are needed to ensure an employee does not become dissatisfied. They do not lead to higher levels of motivation, but without them there is dissatisfaction. Typical hygiene factors are:

- Interpersonal relationships • Work conditions

- Salary • Status

- Security

Motivation Factors

- Motivation factors are needed in order to motivate an employee into higher performance.

These include:

- Achievement • Growth

- Responsibility for task • Interest in the job

Results in Four Scenarios

a. High Hygiene + High Motivation

This is the ideal scenario where employees are highly motivated and have few complaints.

b. High Hygiene + Low Motivation

In this scenario, employees have few complaints but are not highly motivated. The job is perceived as a pay check.

c. Low Hygiene + High Motivation

In this scenario, employees are motivated but have a lot of complaints. This is a situation where the job is exciting and challenging but salaries and work conditions are not up to par.

d. Low Hygiene + Low Motivation

This is the worst situation where employees are unmotivated and have lots of complaints.

Systems Theory

- Unlike the previous theories, the theory places an emphasis on organizations as cooperative systems.

- A system is defined as ‘a set of arrangements of things so related or connected as to form a unit or organic whole’.

- The organization is, therefore, defined as ‘a system of consciously coordinated personal activities or forces’ (Basawanthappa, 2000).

- The systems theory explains that organizations come into existence when there are persons able to communicate with each other who will each contribute an action to accomplish a purpose.
- Managers who understand systems theory recognize how different systems affect a worker and how a worker affects the systems around them.
- A system is made up of a variety of parts that work together to achieve a goal. Systems theory is a broad perspective that allows managers to examine patterns and events in the workplace.
- This helps managers to coordinate programs to work as a collective whole for the overall goal or mission of the organization rather than for isolated departments.

For cooperation to be effective,

The following must be present: • Place where work is done

- Time when work is done
- Person with whom work is done • Things upon which work is done
- Method or process by which work is done
- This helps in understanding the organization in a better way.

Contingency Theory

- This theory asserts that managers make decisions based on the situation at hand rather than a "one size fits all" method.
- A manager takes appropriate action based on aspects most important to the current situation. Managers in a MTC may want to utilize a leadership approach that includes participation from workers, while a leader in the army may want to use an autocratic approach.
- The theory asserts that when managers make a decision, they must take into account all aspects of the current situation and act on those aspects that are key to the situation at hand.
- The continuing effort to identify the best leadership or management style might now conclude that the best style depends on the situation.
- If one is leading troops in Somalia, an autocratic style is probably best; If one is leading a hospital or College, a more participative and facilitative leadership style is probably best

Self-assessment test

Traits leadership theories: ~

- a) Focuses on what leaders do in relational and contextual terms
- b) Emphasizes on the degrees of trust and respect between leaders and followers
- c) Is independent on personal characteristics of followers
- d) Emphasizes that physical and emotional characteristics are critical in inspiring others toward a common goal

The process of working with and through others to achieve organizational objectives is called

Administration

Leadership

Management

Team work

Short response questions

State the four (4) main classification of management theories

Practical test

Using a diagram, demonstrate hierarchy of human needs by Abraham Maslow

References

Louis A. Allen, Management and Organization, McGraw-Hill Kogakusha, Ltd.

Read more: [An Introduction to the Principles of Management - Principles Of Management ~ StateUniversity.com](https://education.stateuniversity.com/pages/cw1ev9e9ib/An-Introduction-to-the-Principles-of-Management.html#ixzz6gbTXQTt2) <https://education.stateuniversity.com/pages/cw1ev9e9ib/An-Introduction-to-the-Principles-of-Management.html#ixzz6gbTXQTt2>

Link: <https://www.google.com/url?sa=i&url=https%3A%2F%2Fwww.verywellmind.com%2Fwhat-is-maslows-hierarchy-of-needs-4136760&psig=AOvVaw0Tm~nXIOioh1DPT3EdHzb1&ust=1608037931433000&source=images&cd=vfe&ved=2ahUKEwjzvOOexs3tAhXa4YUKHVZjC8kQr4kDegUIARDQAQ>

Learning outcome 2: Peri operation Theatre resources

Introduction to the learning outcome

Upon completion of this unit, the trainees should be able to demonstrate knowledge, skills and attitude on Staffing procedures/recruitment procedures, Resource allocation Organization structure/protocol, Communication methods and Ethics in PT

Performance Standard

1. PT activities are identified and classified according to Standard Management Procedures & work place policy
2. PT duties are assigned and allocated appropriate resources according to Standard Management Procedures, WHO and work place policy
3. Responsibilities and authority in PT is delegated and co-ordinated according to Standard Management Procedures, WHO and work place policy

Definition of terms

Job area: Any subunit of a workforce sector, such as a department, job group, job title, etc.

Job description: A written statement detailing the duties of a particular job title.

Job group: Job or group of jobs having similar content, wage rates, and opportunities.

Content/Procedures/methods/illustrations

Human Resource Management

The shift in terminology from 'personnel' to 'human resources' is symbolic of a true change in the field. The term human resource reflects the position that people are a resource that needs to be managed strategically in support of the organisation's mission.

The effective functioning of an organisation depends upon various resources. Human resources or manpower is one of the most vital resources for the labour intensive health institution.

Human resource management can be said to be an integral part of the role of any person who is responsible for the work of others. In this module, human resource management will refer to a group of specialists concerned with increasing the effectiveness of staff performance in operating theatre.

According to Basawanthappa (2002), the main objectives of human resource management are:

Effective utilisation of human resources to enable the achievement of organisational goals.

Establishment and maintenance of an adequate organisational structure and desirable working relationship among staff.

Securing integration of the individual and informal groups with the organisation and thereby ensuring their commitment, involvement and loyalty.

Recognition and satisfaction of individual needs and group goals.

Provision of maximum opportunities for individual development and advancement.

Maintenance of high morals in organisations.

Bearing in mind the objectives covered previously, you will now look at the main aspects of human resource management. These include:

Forecasting Future Manpower Requirements

This involves estimating the institution's demand for labour and matching this with what is available.

Formulating and Proposing Policies

Formulation of human resource policies is done by the human resource management department. The policies have to be agreed upon by top management. The key areas of personnel policy include recruitment and selection, terms and conditions of employment, training and development as well as labour union issues, for example, laid down procedures on how to handle strikes and disputes. These personnel policies are guidelines for behaviour and state how the organisation will respond in relation to employee affairs.

Financial Resource Management

In the previous section, you covered the management of human resources. In this part, you are going to examine financial

resource management. Managing finances is one of the most important roles of a theatre technician manager. Arguments have been put forward that this function should be left to accountants who are specialised in this area. However, if theatre technician do not have the basic skills to enable them manage finances, they will not succeed in providing nursing services. Most of the resources you will be managing are acquired with money.

Financial planning requires that the manager identifies objectives, policies and procedures. The budget is one of the main tools of financial management.

Managing Time

Time is an important resource although in many cases, it is not thought of as such. It is sometimes referred to as one of the M's of management and stands for moments. The other M's being money, manpower and materials. For example, you may recall times when you have wished that a day had more than twenty four hours or that a week had more than

seven days so that you can accomplish the tasks you had set out to do. This often happens when you are not managing time well.

Equipment Used in Theatre

You are now going to look at the equipment that is normally used during an operation.

Ligatures and Sutures

A suture is a stitch or series of stitches used in surgery to bring together living tissues until the normal healing process takes place. A ligature is a suture used for tying blood vessels to arrest bleeding.

There are two types of sutures. They can be absorbable, which means they dissolve in the tissue after some time, for example, catgut. They can also be non-absorbable, which means that the body tissue cannot digest the material used, for example, silkworm gut, nylon, cotton, linen, silk and metal.

The latter must be removed when the wound is healed. Metal clips are also available and are used in neuro-surgery to compress nerve endings, and also on skin incision to give a good grip. Traumatic sutures are used together with a needle for suturing the skin. Ligatures are lengths of suture material used without a needle to tie a blood vessel in order to control or arrest bleeding. Most ligatures are non-absorbable, for example, those made of linen, cotton, silk, polyester, wire and clips. Absorbable include chromic catgut. Metal clips can be used as ligatures.

Cutting needles, which have a sharp edge, cut a crack as they pass, and are used on strong tissues, for example, skin, tendon, muscles.

Surgical Needles

These are made from plated carbon steel or stainless steel. The different parts of a needle are the eye, shaft, and point. The needle is either straight or curved. There are different classes of needles. These include:

Round bodied needles

Cutting needles, which have a sharp edge, cut a crack as they pass, and are used on strong tissues, for example, skin,

tendon, muscles.

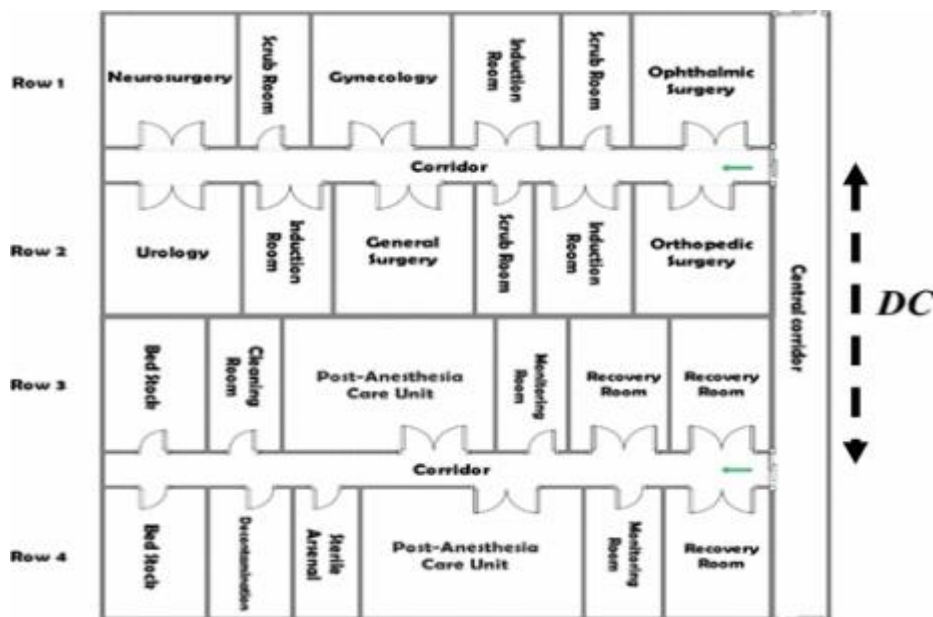
Atraumatic needles

Round bodied needles, which are round and smooth, cause less damage and make a puncture. They are used in delicate tissues and organs.

Cutting needles

Atraumatic needles, which are either cutting or round bodied whose traumatising chance is minimal. These needles have no eye. Suture and needles are made joined-together.

Figure 2: Layout of an Operating Theatre



Adapted from internet

The theatre unit is a block of buildings with a series of rooms leading off a corridor with closed doors, which separate it from the main hospital.

The doors reduce unnecessary movement to and from theatre. A theatre should be built in a central place possibly near an intensive care unit, the surgical wards and other special wards, for example, renal unit and burns unit.

All these units should be in relation to each other, but construction should be separate and independent from all traffic and air movement within the hospital.

A theatre unit is self contained with changing rooms, shower rooms, toilets, anaesthetic room, operating room, cleaning room, at least four beds, sluice room, linen room and sterilising room.

Inside the theatre, the walls, floor and roof are built with labour saving materials for hygiene purposes. It has artificial ventilators, efficient artificial lights and emergency systems for use during power failure. The theatre furnishings and fittings are made of stainless materials for quick and thorough cleaning.

All the trolleys are fitted with non-electricity conducting rubbers to minimise the risk of electric conduction. The doors and corridors are wide and high for easy movement. The ceilings are high enough for proper theatre ventilation.

PHYSICAL LAYOUT THE OPERATING ROOM

An operation theatre is the "heart" of any major hospital. An operating theatre, operating room, surgery suite or a surgery centre is a room within a hospital within which surgical and other operations are carried out.

OT are designed and built to carry out investigative, diagnostic, therapeutic and palliative procedures of varying degrees of invasiveness. Many such setups are customized to the requirements based on size of hospital, patient turnover and may be speciality specific.

The aim is to provide the maximum benefit for maximum number of patients arriving to the operation theatre. Both the present as well as future needs should be kept in mind while planning OT.

OT design

Several basic design types are used in surgical services departments, depending on the age of the facility and the physical design of the areas outside the department. All surgery departments are designed with the idea of controlling traffic patterns and quickly providing each operating room (OR) with the necessary supplies during and after each case, while keeping clean and contaminated traffic patterns separate.

Most surgical suites are constructed according to a variation of one or more basic designs. The basic designs are:

- Central corridor plan – these plans are two types
- Racetrack plan - In this design, the front entrance to each OR is from the outer corridor, and supplies are retrieved through a rear entrance to the room leading to the central-core storage and work areas.
- Hotel plan - In this design, the ORs are situated along a central corridor, with separate clean core and soiled work areas. The primary difference in this plan is that all traffic enters and exits the surgery department through a single entrance or a primary entrance and holding area entrance situated along the same corridor.

- Peripheral corridor - In this design, the front entrance to each OR is from the peripheral corridor, and supplies are retrieved through a rear entrance from the OR leading to the central-core storage and work areas.

- Specialty grouping plan - The "specialty grouping" plan is simply a variation on the hotel or race track plan, in which ORs are grouped by specialty (e.g., neurosurgery, general surgery), each with its own closely associated clean storage areas and, in some cases, each with its own soiled instrument work area.

The design of any surgery department revolves around environmental control, traffic control, and the desire to prevent surgical site infection (SSI). Such factors as the separation of clean and soiled work areas and areas of the department specified as restricted and unrestricted assist in the promotion of this idea. Efficiency is increased with strategic placement of computers, preparation areas, and staff areas.

Different Zones/ Areas

Regardless of the design chosen by the facility, certain principles apply universally. Traffic control follows predetermined traffic patterns that all persons entering the department are expected to follow. The OR department is divided into four zones based on varying degrees of cleanliness, in which the bacteriological count progressively diminishes from the outer to the inner zones (operating area) and is maintained by a differential decreasing positive pressure ventilation gradient from the inner zone to the outer zone.

1. Unrestricted zone or clean area.
 2. Semi restricted zone or sub sterile area
 3. Restricted zone or sterile area
 4. Disposal zone or area
1. Unrestricted/ Clean zone or clean area

- This is the area where hospital personnel, OT personnel, patient's & their attendants can move about in street clothes.

- Connects protective zone to aseptic zone and has other areas also like
- Stores & cleaner room
- Equipment store room
- Maintenance workshop

- Kitchenette (pantry)
- Firefighting device room
- Emergency exits
- Service room for staff
- Close circuit TV control area

1. Semi restricted/ Protective zone or sub-sterile area

• After changing from street clothes, with clean gown, cap & OT slipper, the OT team or personnel are, enter in this area. (Operating room attire is required)

- It includes -
- Change rooms for all medical and paramedical staff with conveniences
- Transfer bay for patient, material & equipment's
- Rooms for administrative staff
- Stores & records
- Pre & post-operative rooms
- I.C.U. and P.A.C.U.
- Sterile stores

1. Restricted/Aseptic zone or sterile area:

- This zone has operation theatres & operation room where operations are done.
- This is a place where staff & patients in street clothes & shoes are not allowed to enter. Scrubbed personnel wear sterile gowns & gloves & get ready for operation procedure. (Operating room attire is required)

1. Disposal zone or area

- Dirty utility area
- Disposal corridor
- OT attire mandatory in this area

Advantages of zoning

- Minimizes risk of hospital infection.
- Minimizes unproductive movement of staff, supplies & patient.
- Increases efficacy of operative team members.
- Ensures smooth workflow.
- Decreases hazards in operating room.
- Ensures proper positioning of equipment's.

Sub areas

- Nurses station or counter - Nurses station is an area where nurses and other health care staff sit behind when not working directly with patients and can perform some of their duties. The station has a counter that can be approached by visitors and patients who wish to receive attention from the nurses. It should be spacious enough to allow two or more personnel to work together.
- Pre-operative check area (reception) - It is the area where OT nursing personnel receive patients. Here patients and his records are checked.
- Holding area - This area is planned for IV line insertion, preparation, catheter / gastric tube insertion, connection of monitors, & shall have O2 and suction lines. Facility for CPR should be available in this area.
- Induction/Anaesthetic room - It should have all facilities as in OT, but there is controversy as to its need. It should provide space for anaesthetic trolleys and equipment and should be located with direct access to circulation corridors and ready access to the operating room. It will also allow cleaning, testing and storing of anaesthesia equipment. It should have sufficient power outlets and medical gas panels for testing of equipment.

Figure 3: Operating theatre room,from internet



Adapted from the internet
<https://www.google.com/url?sa=i&url=https%3A%2F%2Frajnursing.blogspot.com%2F2018%2F09%2Fphysical-layout-operating>

- Post anaesthetic care units (PACU)/Post-operative or recovery room – Room where patients are kept & cared until they are out of anaesthesia & until their vital signs become stable. These should contain a medication station, hand washing station, nurse station, storage space for stretchers, supplies and monitors / equipment and gas, suction outlets and ventilator.
- Staff room – Men and women change dress from street cloth to OT attire; lockers and lavatory are essential; rest room etc. are desirable.
- Sanitary facility for staff- One washbasin and one western closet (WC) should be provided for 8-10 persons. Showers and their number is a matter of local decision. Inclusion of toilet facilities in changing rooms is not acceptable; they should be located in an adjacent space.
- Offices – for staff nurse and anaesthesia staff- The office should allow access to both unrestricted and semi-restricted areas as frequent communication with public is needed.
- Rest rooms- Pleasant and quiet rest for staff should be arranged either as one large room for all grades of staff or as separate rooms; both have merits. Comfortable chairs, one writing table, a book case etc., may be arranged.
- Laboratory – Small laboratory with refrigerator for pathologist to be arranged.
- Seminar room- Since staff cannot leave an OT easily, it is better to have a seminar room within the OT. Intra-departmental discussions, teaching and training sessions for staff (with audio-visual aids) may be conducted here.
- Store room-

- These rooms should have steel cabinets & separate for specific items.
- These cabinets should be spacious enough to accommodate various items such as sterile linens, sterile trays, sterile drums with sponges, dressings, cotton swabs, bandages, sterile rubber sheets, catheters, sutures, syringes, infusion & transfusion sets.
- A separate store room is needed for unsterile, clean articles such as linen, OT dress items, stainless steel equipment's
- The cabinets of store rooms should be made up of such materials which can be cleaned & washed without being damaged.
- There should be bigger store room for equipment's such as monitoring devices, suction apparatus, O2 cylinder, trolleys, wheel chairs, stretchers, defibrillator, pace makers, bed urinals, OT slippers etc.
- Scrub room- This is planned to be built within the restricted area. This should be spacious. Sometime s two or three operation theatres can share one scrub facility. Scrub area should have deep & wide sink to avoid splash of water on the surrounding area. There should be facilities for running water. The taps should be such that they could be opened or closed by foot pedals or they have long handles that can be operate by elbows.
- Waiting room – Patient's attendants or family member's waiting room with attached toilet & drinking water facility.
- Visitor's gallery - Gallery from where the visitors can see the operative procedures being performed.
- Utility Room - This is also called clean up room where instruments, articles, gloves are received following surgical procedures. Here all these are cleaned, washed & wiped dry & then wrapped, stored & sent for sterilization.
- Disposal or sluice room - This room receives all soiled linen & dressing. Here all disposable items are separated according to biochemical waste management code & then packed in leak proof containers. The soiled linens are sent to the laundry. The OT set up should be such that it should be possible to dispose off soiled items without taking them through sub-sterile or sterile areas.

OPERATING ROOM

NUMBER & SIZE

- The number & size can be as per the requirement.
- The standard OT should be rectangular or square in shape and similar in design.
- It should be 20×20×10 feet in size so it provides a floor space of 400 Sq. feet approx.

- It should be spacious enough to allow free movement of personnel, trolleys, stretchers, wheel chairs, monitoring devices, portable x-ray machines etc.
- Cardiac or neurosurgery OT should be bigger in size of 20x30x10 feet with 600 sq. feet floor space.
- OT for endoscopy or minor surgery can be of small size of 18x18x10 feet with a floor space of 324 sq. feet.

DOORS

- Main door to the OT suite has to be of adequate width (1.2 to 1.5 m).
- The doors of each OT should be spring loaded flap type, but sliding doors are preferred as no air currents are generated.
- All fittings in OT should be flush type and made of steel.

SURFACE / FLOORING

- The surface / flooring must be slip resistant, strong with minimum joints.
- It should be easily washable, free as possible joints, seams so that they do not permits adherence of bacteria or dust particles containing microorganism.
- It should be able to withstand repeated washing with germicidal agents.
- It should be able to absorb sound.
- The colour of the flooring should be such that if a needle is dropped on the floor it is visible.

WALLS

- The walls, ceiling & floor surfaces should be made of hard fire resistant, smooth, non-porous material.
- It should be light in colour (light blue or green) and washable paint should be ideal.
- Colour of paint should allow reflection of light and yet soothing to eyes.
- Adequate electric points should be available on the wall at < 1.5 m height from the floor.

SCRUB STATION

- For sterile hand wash
- Types: Stainless/ Granite/Marble/ Cement

- Gradient of basin forward and downwards
- Gradient towards outlet
- Hot water mixing: automatic or manual
- Non splashing taps; possible to change direction
- Elbow/ Foot or Infra-Red operated taps
- Soap dispenser: Manual/automatic
- to be planned for at least for 2-3 persons in each OT.

VENTILATION, TEMPERATURE & HUMIDITY

- The OT Require Efficient ventilation that will control temperature and humidity in OT and dilute the contamination by microorganisms and anaesthetic agents.
- The ventilation in OT should follow the principles that the direction of airflow should take place from the sterile to the clean zone & from the clean zone to the less clean areas.
- There should be no open window in the OT and sliding doors of the OT should be open from the OT towards the sub sterile zone.
- There should be no interchange air movement between one OT and another.
- There should be no circulating fan or cross ventilation to prevent airborne contamination of the surgical wound.
- An effective ventilation system is necessary to exchange the air. This air is filtered with an efficient air filtering system.
- There are two types of air conditioning systems: recirculating and non recirculating.
- The circulating system takes some or all of the air, adjusts the temperature and circulates air back to the room.
- Non-recirculating systems heat / cool the air as desired and convey it into the operating room with ideally 20-air exchange per hour. Air is then exhausted to outside. Anaesthetic agents in the OT air are also automatically removed. These are thus ideal but are expensive.
- The broad recommendations include:

- 20-30 air exchanges / hour for recirculated air
- Only up to 80% recirculation of air to prevent build-up of anaesthetic and other gases
- Ultraclean laminar airflow – the filtered air delivery must be 90% efficient in removing particles more than 0.5m m.
- Positive air pressure system in OT: It should ensure a positive pressure of 5 cm H₂O from ceiling of OT downwards and outwards, to push out air from OT.
- Relative humidity of 50-60% to be maintained
- Temperature between 18o-24oC. Temperature should not be adjusted for the comfort of OT personnel but for the requirement of patient, especially in paediatric, geriatric, burns, neonatal cases etc.

LIGHTING

Figure 4 showing the diagram of surgical lights



Adapted from the internet

<https://www.google.com/url?sa=i&url=http%3A%2F%2Fwww.surgicalledlights.com%2Fsale-11200111-5000k-220v-led-operation-theatre-lights-140000-lux-surgical-operating-light>.

The general room illumination the OT is provided by fluorescent lamps (surface ceiling mounted) to produce even illumination of at least 500 Lux at working height, with minimal glare are preferred.

- This contrast should be maintained in corridors and scrub areas, as well as in the room itself, so that the surgeon becomes accustomed to the light before entering the sterile field.
- To minimize eye fatigue, the ratio of intensity of general room lighting to that at the surgical site should not exceed 1:5, preferably 1:3.
- Colour and hue of the lights also should be consistent.
- The surgical or operation light should be an overhead ceiling mounted unit.
- The overhead operating light must:
 - Overhead light should be near daylight in colour and shadow less and give 25000-125000 Lux of light (50000 to 100000 Lux at the centre and at least 15000 Lux at the periphery).
 - Give contrast to the depth and relationship of all anatomic structures. The light may be equipped with an intensity control mechanism. The surgeon may ask for more light when needed therefore a reserve light should be available (e.g. a mobile operation light).
 - Provide the diameter light pattern of a focus appropriate for the size of the incision. These are adjusted with controls mounted on the light fixture.
 - Be freely adjustable to any position or angle. Most overhead operating lights are ceiling mounted on mobile fixtures. It can be positioned so that light is directed into a single incision or two concurrent operative sites.
 - Be spark-proof where anaesthetic gases are used.
 - Produce minimum heat to prevent injury to exposed tissues, to ensure the comfort of the sterile team, and to minimize airborne microorganisms.
 - Be easily cleaned. Tracks recessed within the ceiling virtually eliminate dust accumulation. The radiant heat produced by the light should raise the tissue temperature not more than 2°C.

EMERGENCY SIGNAL

- The OT management committee should consider installing an alarm system in case of a life-threatening emergency.
- The activation of this alarm will save valuable time to mobilize the emergency designate staff and direct them to the site of the emergency.

OTHER FACILITIES OF OR

- Electric generator - OT department should have electric generator to ensure uninterrupted electricity supply in case of electricity failure.
- O2 supply & suction system - OT should have piped in O2 supply through central O2 supply system from a central source. There should also be central suctioning through piped in suction arrangement.
- Music system - Operation theatre can also have a soft music system. Music creates a pleasant environment for patients & staff. Music also provide diversion of the patient's mind. Music can provide relaxation to pts who undergo surgery under local, regional or spinal anaesthesia. It also provide a soothing atmosphere & decreases tension & fatigue of the OT staff.
- Computer terminal - The modern OT construction also provide a surface area for the computer terminal in each OT. These computers are being used to record & maintain patient information.

Resource mobilization

What is Resource Mobilization and Why is it so Important?

What is resource mobilization in theatre?

Resource mobilization refers to all activities involved in securing new and additional resources for your organization. It also involves making better use of, and maximizing, existing resources. Resource mobilization is often referred to as 'New Business Development'. The figure below shows how New Business Opportunities – which are intended to mobilize resources – form part of an organization's overall functioning.

Why is resource mobilization so important?

Resource mobilization is critical to any organization for the following reasons:

1. Ensures the continuation of your organization's service provision to clients
2. Supports organizational sustainability
3. Allows for improvement and scale-up of products and services the organization currently provides
4. Organizations, both in the public and private sector, must be in the business of generating new business to stay in business

What is meant by sustainability?

Although sustainability is often identified with having sufficient funds to cover an organization's activities, it is actually a broader concept. There are three fundamental streams of sustainability: institutional, financial and programmatic. Each is vital to the survival of an organization. Below are the definitions of these three areas of sustainability:

Programmatic sustainability. The organization delivers products and services that respond to clients' needs and anticipates new areas of need. Its success enables expansion of its client base.

Institutional sustainability. The organization has a strong, yet flexible structure and accountable, transparent governance practices. Its structure and good governance allows it to respond to the shifting priorities of its supporters and to new responsibilities toward its clients, while creating a positive work climate for its staff.

Financial sustainability. The organization draws on various sources of revenue, allowing it to support its ongoing efforts and to undertake new initiatives.

Staffing Process

Staffing involves hiring and retaining staff in an organisation.

The staffing process involves a situation analysis, looking into what you want and whom you can get. It includes:

- Recruitment
- Selection
- Induction
- Scheduling

Recruitment

This is the first part of the process and it involves filling a vacancy. It includes the examination of the vacancy and consideration of sources of suitable candidates. The sources could be internal, that is, from within the organisation, or external.

Selection

This is the next stage, it includes assessing the candidate by various means, for example, interviewing and making a choice between candidates to be followed by an offer of employment. If no suitable candidates are found, the post is advertised again with a wider scope or catchment area. This may include advertising abroad.

During selection, at least the three best candidates are chosen.

In case the best candidate is not in a position to take the job, then the second best is approached until the vacancy is filled.

Scheduling (Duty Roster)

Figure 5: Sample of duty roster

STAFF DUTY CHART - http://www.setupmyhotel.com							
December	Mon 03	Tue 04	Wed 05	Thu 06	Fri 07	Sat 08	Sun 09
Occupancy %							
No Of Arrivals							
No Of Departures							
No Of In-House							
FRONT OFFICE AGENT							
FOA 1							
FOA 2							
FOA 3							
FOA 4							
CASHIER 1							
CASHIER 2							
CASHIER 3							
BELL DESK							
STAFF 1							
STAFF 2							
STAFF 3							
LEGEND							
07:00 - 15:00	M						
15:00 - 23:00	A						
23:00 - 06:00	N						
Weekly OFF	WO						
Comp OFF	CO						
Public Holiday	PH						

Prepared By :

Approved By:

www.setupmyhotel.com

From internet
<https://www.google.com/url?sa=i&url=https%3A%2F%2Fsetupmyhotel.com%2Fformats%2Ffo%2F181-hotel-staff-duty-roster-chart>

After the orientation is done, the employee is assigned tasks to be performed. Work schedules and time off should meet organisational goals with fairness and equity among personnel. A schedule should adhere to the following:

- Policies, standards and practices of the organisation on the hours worked by professional and support staff.
- Appropriate ratio or balance between professional and support staff.
- Continuity of nursing services and twenty four hour coverage.
- Approved budget.
- Consideration of vacations.
- Allowance of adjustment in case of illness, emergencies or changes in patient care needs.

- Individual needs of staff.

Staff members should be informed of their work schedules at least one to two weeks in advance.

The schedules may be either centralised or decentralised.

A centralised schedule is controlled by the chief nurse, matron or a designate who develops a master plan for nursing personnel in the hospital. One of the advantages of this method is that it provides an overall picture of the staffing situation in the organisation. However, it denies the manager the right to make scheduling decisions based on workers' abilities and needs. In the decentralised schedule, the middle and lower levels of management determine scheduling, for example, ward in charges. The manager is, therefore, more accountable at their level and has a greater control of activities.

Factors Affecting Staffing

There are many variables that may affect staff numbers and placement.

These include:

The need to provide nursing services

- Staff factors, for example, job descriptions, education level of staff, experience and expectations from the organisation.
- Patient factors, including variety of patient conditions, length of stay, the patient population, care needs and fluctuation in numbers.
- Health care organisation factors, including policies and procedures, financial resources available, number of beds per unit, staffing norms, issues of professional coverage and nursing assignment systems.

Since each setting is unique, there is no guide that can stipulate the correct number of personnel needed to provide quality care. However, systems have been developed for guidance, for example, the patient classification system which is a method of grouping patients according to the amount and complexity of their nursing care requirements. In this case, patients are grouped according to the nursing time, effort and ability required to provide care.

Organising

Nurses work in organisations. Nurse managers are expected to organise their work activities based on the staffing patterns. Organising is the process of grouping the necessary responsibilities and activities into workable units, determining the lines of authority, communication, developing patterns of coordination and giving feedback. By organising, you are attempting to answer the question:

How will the work be divided and accomplished?

To answer the question, the manager must define groups and assign duties. There are certain basic issues to consider:

- Set up structure, that is, the structured aspects of the organisation that must be set up to indicate activities to be performed and lines of responsibility

and authority.

- Develop procedures and policies.
- Determine requirements and decide how duties will be performed.

Resource allocation

Financial Resource Management

In the previous section, you covered the management of human resources. In this part, you are going to examine financial resource management. Managing finances is one of the most important roles of a nurse manager. Arguments have been put forward that this function should be left to accountants who are specialised in this area. However, if nurses do not have the basic skills to enable them manage finances, they will not succeed in providing nursing services. Most of the resources you will be managing are acquired with money.

Financial planning requires that the manager identifies objectives, policies and procedures. The budget is one of the main tools of financial management.

Budgeting

In simple terms, budgeting involves managing money and how it is used to maintain other resources.

A budget is a means of control, which reflects the plan against which actual performance is measured. It can also be defined as a means of checking the progress made in keeping the expenses and costs in compliance with an organisation's financial plans. Budgeting is the process of planning and controlling future operations by comparing actual results with planned budgetary expectations.

The following are some of the purposes of a budget:

- To avoid spending more than an organisation can afford
- To aid in planning and controlling
- To assist in assessing the financial requirements of the institution

- To indicate the areas in which money raised or received will be spent
- To facilitate comparison of actual performance with budgeted targets

In order to draw up a meaningful budget, the hospital should have a clearly defined organisational structure with responsibilities defined and assigned. Personnel at all levels of management should participate in budget development. The personnel involved should have an understanding of the ideas and financial goals of the hospital.

There must be an adequate system, which provides reliable financial and statistical information to the responsible people. The budget should allow enough freedom to accomplish departmental objectives and must be flexible enough to allow for unpredictable expenditures.

There are two main types of budgets, which the nurse may be involved

in preparing:

- Operational or Annual Budget
- Capital Budget

Operational or Annual Budget

This is the organisation's statement of expected revenue or income and expenses for the coming year. It coincides with the fiscal year. The fiscal year is a specified twelve month period during which the operational and financial performance of the organisation is measured. It usually coincides with the calendar year, that is, it runs from January to December or may follow another time frame, for example, the government calendar, which runs from July to June.

The operational budget includes the accumulated estimates of operating revenues and expenses. In nursing management, the revenue is derived from patient fees. Actual payment is generated by a given service or procedure.

The expenses consist of salary and non salary items, for example, personnel emoluments and education, including in service training, on the job training, educational leave and travel scholarships. They may also include personnel uniforms, books, periodical subscriptions, laundry, medical equipment and maintenance, drugs and pharmaceuticals, non pharmaceutical supplies, legal and professional fees and stationery.

The expenses should be comprehensive and thorough. Budgets are prepared by the heads of each ward or department in consultation with staff. Thereafter, the matron, chief nurse or director of nursing services compiles the nursing division budget. In some cases, the director of nursing services prepares a budget for the whole of the nursing division.

As you can see from the figure, the income and expenditure parts of the budget have the same totals, that is, they balance. A budget that has a higher income than expenditure is said to show a surplus while the one with a higher expenditure shows a deficit. If there is

a deficit in the budget, it means you have spent more money than you have. This may require you to borrow money or be indebted to your suppliers.

Capital Budget

This is the second type of budget. It outlines the need for major equipment or physical changes in the organisation requiring large sums of money, for example, physical renovations, new construction and new or replacement equipment. Organisations define capital items based on certain criteria, for instance, the item must have an expected performance of a minimum of one year or more and exceed a certain shilling value, for example, fifty thousand. The criteria vary from one hospital to another.

Requests for capital items are usually made on a special form or letter, accompanied by a written justification for the item. If there are a large number of items requested, a priority list should be set.

Where possible, the period within which items are needed should

be specified. Capital costs should consider long

term goals and must complement the organisational objectives.

All nurse managers are involved in the budgeting process.

Preparation begins several months before the end of the fiscal year to allow time for careful preparation. In a decentralised system, budget preparation has several steps:

- Review of policies, standards and objectives.
- Top level management projections for the future and preparation of guidelines.
- Middle level nurse managers prepare the annual budget.
- The administrator of nursing services, finance director or manager reviews the budget.
- Budget is accepted or modified.
- Budget is implemented and regularly evaluated.

Capital Budget

As you can see from the figure, the income and expenditure parts of the budget have the same totals, that is, they balance. A budget that has a higher income than expenditure is said to show a surplus while the one with a higher expenditure shows a deficit. If there is a deficit in the budget, it means you have spent more money than you have. This may require you to borrow money or be indebted to your suppliers.

Managing Materials

So far you have managed human resources and money. The other resource that a manager is concerned with in the organisation is the materials. Materials refer to drugs, supplies and equipment needed by the nurses and other health personnel to deliver services.

For quality and efficient services, the materials must be provided at the right place, at the right time and in the right quantity.

Materials are essential if the health care institution is to achieve its objectives. The nurse manager should know the policies and procedures that relate to materials management. You should be familiar with the following activities:

Demand Estimation

Since a large quantity of materials is used in hospitals, you need to identify your requirements or needs beforehand.

Procurement

Having developed your list of requirements, the next step is procurement or ordering.

Some institutions have laid out specific rules and regulations regarding procurement. This is aimed at reducing wastage and maximising the value of money.

Receipt and Inspection

The materials received should be subjected to either physical or chemical inspection. This ensures that you received the right quality and quantity of materials as requested by the organisation.

Inventory Control

Adequate quantities of materials should be available

whenever required. Close supervision of movement of materials or consumption rate is a good tool for proper control.

Storage

The materials should be placed in a store within or near the institution. The store should be of adequate size to accommodate all the different materials required, for example, fridges, shelves and cupboards.

The materials should also be stored in appropriate conditions, for example, temperature, light as specified by the manufacturer.

It is, therefore, important that the nurse manager adopts the proper procedures for materials management.

Managing Time

Time is an important resource although in many cases, it is not thought of as such. It is sometimes referred to as one of the M's of management and stands for moments. The other M's being money, manpower and materials. For example, you may recall times when you have wished that a day had more than twenty four hours or that a week had more than seven days so that you can accomplish the tasks you had set out to do. This often happens when you are not managing time well.

The demands on time as a manager and a health care provider fall into three categories (Nduba 1999):

- Tasks that have to be performed. These relate to key responsibilities, for example, clinical or administrative duties.
- Tasks that other people pressurise you into doing. Some of these are important and must be done while others you do because you do not want to say no.
- Tasks that you perform because you want to.

These are usually out of choice and include tasks that someone else could do just as well, for example, attending a nurses' association meeting.

Time Waster

This is something that prevents a person from accomplishing a job or achieving a goal. Some of the common time wasters include:

- Interruptions such as telephone calls and drop in visitors.
- Lack of clear cut goals, objectives and priorities.
- Meetings both scheduled and unscheduled.
- Lack of daily or weekly plans.
- Lack of self discipline.
- Failure to delegate.
- Ineffective communication.
- Inability to say no.

Principles of Time Management

The following are techniques that you can use to deal with time management constraints.

Goal Setting

The nurse manager sets both organisational and

personal goals. The goals are either short or long term and provide direction and vision for actions as well as time frames in which activities will be accomplished.

Time Analysis

The manager should conduct a survey of how they spend a day. Reviewing the daily schedule and keeping it accurate may demonstrate how time is used.

Priority Setting

The nurse manager identifies time frames for achievement of goals. The 'to do' list should be prioritised by classifying activities as 'one' for urgent, 'two' for not urgent but important and 'three' for less important.

Delegation

The nurse manager can delegate those activities that can be effectively handled by juniors.

Controlling Interruptions

The nurse manager should identify causes of interruption and plan to reduce them. One way of doing this is incorporating some of these interruptions into planned and scheduled activities.

Nurse managers must use time wisely to accomplish everything that is expected of them. This takes a lot of planning.

By now you should be able to manage resources within the organisation. However, in the current health care system, there are a lot of changes including social, technological and educational, which a nurse manager should be able to respond to and manage. According to the theory of evolution, plants and animals have survived countless changes in the

earth's environment. For example, the cockroach has been around for three hundred million years and the shark two hundred million years. This is because they have learnt how to adapt to their environment. They have learnt how to survive. The strong survive and the weak die, it is survival for the fittest.

Change has always been, and will always be, a part of us. During your lifetime you must have experienced change in one form or the other, such as moving from one workstation to another, acquisition of new skills or introduction of new methods of nursing care delivery.

Change is the process of making something different from what it was. Change is inevitable and nurse managers should understand and anticipate the reactions to change.

Figure 6: Organization structure/protocol Organizational Chart for Division



Figure 7: advantages of grouping OT as a management system

Communication methods

Organisational Communication

In units one and two of module one, you covered the various principles of communication. Please refer to your notes on this topic, as they will be useful for the following topic. In this section, you will specifically focus on organisational communication. There is no organisation that can exist without communication.

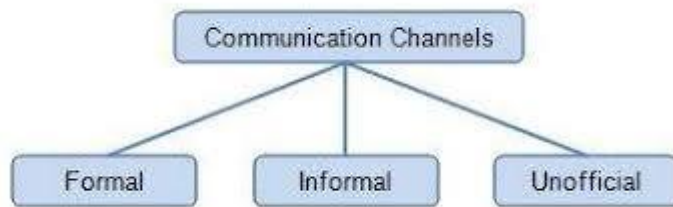
Communication is referred to as the lifeblood of any organisation. Communication refers to the need to create

common understanding. It is, therefore, the process of transmitting ideas and information from one person, the sender, to another or group of people, receivers.

Organisational communication is the exchange of information within the organisation. While employee communication is the responsibility of the manager, the manager's work may be supplemented through some already established means of communication within an organisation.

The manager is charged with the responsibility of passing information through various channels.

Figure 8: Showing communication channels



Formal and Informal Communication

The formal communication system is mainly used. It is based on a chain of command from the top of the organisation to the bottom. This is used for all official messages including directives, procedures, policies, job instructions and so on. An informal communication system can also be used. This is usually oral and not crucial to the functioning of the organisation.

It originates from informal groupings, for example, associations or

welfare groups. In most cases, messages are transmitted through the 'grapevine' (also known as rumours) and sometimes the communication is distorted or groundless. Such types of communication can cause fear and anxiety among staff and the manager should be aware of its existence and reinforce stability using more formal methods of communication.

You will now look at some of the different types of formal organisational communication.

Vertical Flow

The manager is responsible for passing information through vertical channels of communication, which includes both a downward and an upward flow of communication.

A two way exchange of information is essential in organisations and managers must ensure that there is feedback at all levels.

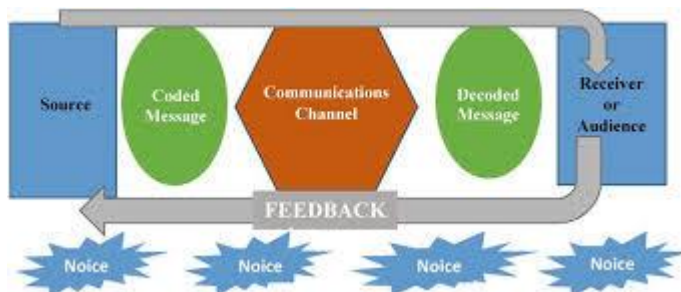
Downward Communication

Most information within organisations moves from the top downward, that is, from the top levels of management to employees. This may be through face to face communication or written materials, for example, memos or circulars.

The purpose of this form of communication is mainly to inform employees of their responsibilities, commitment to the objectives of the organisation and any other relevant information. Barriers to this form of communication may include:

- The manager may withhold information
- The employee may fail to understand the message
- The employee may get information but this may not be relevant to their needs

Figure 9: Showing Upward Communication



From the internet

This is the flow of information from the bottom up or from the subordinates to the supervisor. Ideally information should pass freely up the chain of command so that management gets the feedback needed to evaluate results and initiate improvements. Barriers to upward communication may include:

- If employees feel management is not interested in their ideas, they will not offer them.
- At times, bad news gets blocked and is not communicated to the top management.
- Delays in relaying messages.

Ethics in PT

Legal and Ethical Issues In perioperative theatre technology

You will start by looking at the term 'legal'. The dictionary defines the word legal as 'required' or 'permitted by law'. Therefore, when we talk of legal aspects in theatre technology, we are referring to what the law requires us to do in the theatre before, during and after the operation. In your clinical practice as a nurse, you may have participated in helping a patient who was to undergo an operation. Can you remember what preoperative care was required before the patient could go for the operation?

Preoperative Care

The preoperative care requirements are:

1. You should make sure that the surgeon explains clearly to the patient what will happen to them.
2. The surgeon should obtain an informed consent from the patient or parent/guardian/next of kin for those under age or not in a position to sign (e.g. unconscious person).
3. The nurse ensures that the patient has signed an informed consent, after the surgeon has explained the advantages and outcomes of the operation.
4. Make sure that the patient observes a 'Nil by oral' rule. The fasting should usually start six hours before the operation.
5. Blood works: All should be within the acceptable ranges e.g. full Haemogram including HB, urea, electrolytes and creatinine.
6. The patient should be counselled and reassured especially those receiving operations such as amputation, or mastectomy.
7. The site to be operated on should be shaved of hair and cleaned with warm soapy water, to reduce the bacteria on the patient's skin. The area shaved should be larger than the incision site.
8. Catheterisation and IV cannula insertion may be necessary depending on the surgery.
9. Observations of vital signs, urine testing for sugars, proteins and acetone should be done.
10. The receiving area nurse should confirm that the above preoperative measures have been taken by the ward nurse in order to allow the patient in theatre. Some of the preparations form part of the legal requirements before surgery.

In unit one of this module you covered nursing ethics where the importance of confidentiality in nursing practice was stressed. This is another legal requirement. In the definition of legal, the term 'permitted by law' implies that you can only carry out patient care within what the law permits you to do. Therefore, the law gives the patient seeking medical, surgical and nursing care, rights under which they are to be managed.

The dictionary defines rights as any claim that is morally just or legally granted as allowable or due to a person. This brings you to the term 'legal rights of an individual during theatre nursing'.

When it is said that a person has the legal right to informed consent, what is meant is that they must be given information regarding the type of operation to be performed, why it is necessary, and its effects both bad and good, before being requested to sign the consent form for the operation.

However, you must also be aware of the fact that not all patients are given all the information regarding the type of operation they are to undergo. Before you give such information, you need to take factors such as age and level of education into consideration.

No matter how impossible it is to communicate this information to a patient, it would be against the law and their individual rights to ask them to sign the consent form for an operation when they have not fully understood the implications.

It is on this basis that those below the legal age of adulthood (18 years in Kenya) are not legally bound to sign the consent form. It is signed by the parents/guardians on their behalf. In the same way, consent for the mentally ill is sought from their parents/guardians/relatives. It is also important to note that consent for an operation should be obtained from the patient before they are pre-medicated, as pre-medication drugs have the potential of affecting their reasoning capacity, hence making consent signed not legally binding.

Having looked at confidentiality and informed consent, it is important to mention that the custody and security of the patient before, during and after operation is vested in the theatre team. It has already been implied earlier that by signing the consent form, the patient takes some responsibility for the whole loss of life or part of their body. However, this does not take away the responsibility of the theatre team to ensure the security of the patient's life during the operation.

The legal aspect in theatre nursing involves the care of the patient from the time the patient is accepted in theatre, until they are handed over back to the ward.

For these reasons, the following procedure should be adhered to:

1. Any patient going to theatre must be properly prepared preoperatively.
2. The patient must sign an informed consent, obtained by the surgeon.
3. The patient must be protected from any harm, falls or eventuality,
during the stay in theatre.
4. Confidentiality must be observed regarding the patient.
5. Measures must be taken to ensure that the patient taken to theatre is the right one for the intended operation.

6. The items to be used for the operation must be counted and recorded before and after operation to prevent loss of swabs, tubes, blades, forceps, abdominal packs and any instrument used.
7. Theatre technician must know where the exits are, for use in case of an emergency.
8. Sockets in theatre should be covered during scrubbing to prevent risk of conducting currents. They should also be one meter or more above the floor level.
9. All electrical machines must be checked to ascertain optimum function before use on the patient.

Ethical Issues

Ethics have always been an integral part of perioperative theatre technology. Throughout your practice you have applied the nursing code of ethics. The ethics provide the standards for professional behaviour in terms of what is expected of theatre technician. They are based on the principles of what is right or wrong. The nurses' code of ethics serves as a self regulatory mechanism and a source of guidelines for individual behaviour and responsibility.

As mentioned earlier, the nurse manager plays a significant role to ensure that the law and ethics are applied appropriately in theatre technician practice. The following are some of the roles and functions expected of them:

- Serves as a role model by providing care that meets and exceeds accepted standard of care.
- Ensures that their conduct is as per the perioperative theatre technology code of ethics.
- Makes sure that their staff confine themselves to the scope of nursing practice according to registration, licensure and enrolment.
- Delegates to subordinates wisely.
- Provides educational and training opportunities for staff on legal issues.
- Is aware and makes staff aware about their own values and basic beliefs about the rights of human beings.
- Advocates for clients, subordinates and the profession.

Written assesment

Short answer responses

1. explain the proper theatre layout

2. discuss different communication channels

3. list all the resources found in the theatre

5. outline the time management in the theatre

6. explain staff recruitment process

Practical task

Develop a duty roster for five perioperative theatre technicians

Materials

Paper

Pen

Ruler

Pencil

names

References

Louis A. Allen, Management and Organization, McGraw-Hill Kogakusha, Ltd.

Figure 1 layout:
<https://www.google.com/url?sa=i&url=https%3A%2F%2Fwww.researchgate.net%2Ffigure%2Ffirst-case-modified-OT-layout>

Figure 2 operating room

<https://www.google.com/url?sa=i&url=https%3A%2F%2Fajnursing.blogspot.com%2F2018%2F09%2Fphysical-layout-operating->

figure 3 operating room lighting

<https://www.google.com/url?sa=i&url=http%3A%2F%2Fwww.surgicalledlights.com%2Fsale-11200111-5000k-220v-led-operation-theatre-lights-140000-lux-surgical-operating-light>.

Figure 4 duty roster

<https://www.google.com/url?sa=i&url=https%3A%2F%2Fsetupmyhotel.com%2Fformats%2Ffo%2F181-hotel-staff-duty-roster-chart>

Learning outcome 3: Direct Operation Theatre resources

1.2.2 Introduction to the learning outcome

This unit describes the competencies required direct operation theatre resources. This learning outcome involves, Interpersonal skills, employment Act, work place policies, law and Labour regulations and Leadership skills

1.2.3 Performance Standard

1. PT activities are supervised in accordance to Standard Management Procedures and work place policy
2. PT staffs are motivated in work as per the labour laws and work place policy
3. Guidelines in PT are given according to Standard Management Procedures, WHO and work place policy

Definition of key terms

Interpersonal skills -are the *skills* we use every day when we communicate and interact with other people, both individually and in groups.

Conflict- is serious disagreement and argument about something important. If two people or groups are in **conflict**, they have had a serious disagreement or argument and have not yet reached agreement

Content/procedures/methods/illustrations

Interpersonal skills

What are interpersonal skills, and why are they important in the workplace? Interpersonal skills, also known as people skills, soft skills, or emotional intelligence, are related to the way you communicate and interact with others.

When employers are hiring, interpersonal skills are one of the top criteria used to evaluate candidates. Regardless of the type of job you have, it's important to be able to get along well with coworkers, managers, customers, and vendors.

Strong interpersonal skills are essential for succeeding in today's workplace.

What Are Interpersonal Skills?

Interpersonal skills are sometimes called employability skills. The word “employability” is a tip-off about the importance of interpersonal skills: they’re so crucial that hiring managers really don’t want to hire candidates without them.

Many careers require consistent, if not constant, interaction with other people. This is true even for jobs that would seem to favor introverted personalities and independent work styles. For example, even if you’re a software engineer, writer, or statistician, you still need to be able to communicate and collaborate with your team.

It’s important to emphasize your interpersonal skills in your cover letter and resume, and then back up those claims with your behavior during job interviews.

Even if you excel at the technical aspects of your job, employers won’t want to hire you if it seems like you’d be a disaster to work with.

Types of Interpersonal Skills

Communication

One of the most important interpersonal skills in any job is communication. Whether you work in IT, customer service, construction, or any other industry, you will need to be able to communicate clearly and effectively with others both verbally and in writing. Some jobs also require skills in effective public speaking.

- Nonverbal communication
- Public speaking
- Verbal communication

Conflict Management

Whether you are a manager or an employee, you will likely need to resolve conflicts at some point in your job. This might involve solving an issue between two staff members, between yourself and a colleague, or between a client and your company. You will need to be able to listen fairly to both sides and use creative problem solving to arrive at a solution.

- Conflict resolution
- Constructive criticism
- Counseling
- Mediating
- Problem solving

Empathy

Part of being a good manager, employee, or colleague is the ability to understand and show empathy for others. If a customer or colleague calls with a complaint, for example, you will need to listen thoughtfully to the person's concerns and express compassion for their issue. Empathy is an important skill that will help you get along with everyone in the workplace.

Caring, compassion, diplomacy, diversity, helping others,
kindness, patience, respect, sensitivity and Sympathy

Leadership

Even if you are not a manager, it is important to have some leadership experience and ability. Leadership requires being able to motivate and encourage others and help a team achieve success.

Listening

Listening is a skill that goes hand in hand with good communication. While you need to be able to express your own ideas, you also need to thoughtfully listen to the ideas of others. This will help your clients, employers, colleagues, and employees feel respected and valued.

- Active listening
- Curiosity
- Focus
- Inquiry

Negotiation

Negotiation is an important skill for many positions. Depending on the specific job, it might involve creating formal agreements (or contracts) between clients or helping colleagues solve a problem and determine a solution. To be a good negotiator, you must be able to listen to others, use creative problem solving, and arrive at an outcome that satisfies everyone.

- Negotiating
- Persuasion
- Research

Positive Attitude

Employers want to hire employees who make the office a brighter place. They want people with a friendly, positive demeanor. This doesn't mean you have to be the most social person in the office, but you must be willing to develop some sort of a positive rapport with your colleagues.

- Behavioural skills
- Developing rapport
- Friendliness
- Humour
- Networking
- Social skills

Teamwork

Even if your job involves a lot of independent work, you still need to be able to collaborate with others. Teamwork involves several of the skills already mentioned: you need to be able to listen to others, communicate your own goals, motivate your team, and resolve any conflicts that may arise.

- Collaboration
- Group facilitating
- Team building
- Teamwork

Work place policies

Workplace Policies and Procedures Checklist

Explore this page

- What is a workplace policy?
- Benefits of having workplace policies
- Developing and introducing workplace policies
- Policy checklist
- Types of workplace policies

What is a workplace policy?

A policy is a statement which underpins how human resource management issues will be dealt with in an organisation. It communicates an organisation's values and the organisation's expectations of employee behaviours and performance.

Workplace policies often reinforce and clarify standard operating procedure in a workplace. Well written policies help employers manage staff more effectively by clearly defining acceptable and unacceptable behaviour in the workplace, and set out the implications of not complying with those policies.

A workplace policy consists of a statement of purpose and one or more broad guidelines on action to be taken to achieve that purpose. The statement of purpose should be written in simple terms, free of jargon. The length of the policy may vary depending on the issue it addresses.

A policy may allow discretion in its implementation and the basis of that discretion should be stated as part of the policy. A policy may also be required where there is a diversity of interests and preferences, which could result in vague and conflicting objectives among those who are directly involved.

Not all workplace issues require a policy. Many routine matters can be dealt with through simple workplace procedures and processes being put in place.

Benefits of having workplace policies

Well-written workplace policies:

- are consistent with the values of the organisation
- comply with employment and other associated legislation
- demonstrate that the organisation is being operated in an efficient and businesslike manner
- ensure uniformity and consistency in decision-making and operational procedures
- add strength to the position of staff when possible legal actions arise
- save time when a new problem can be handled quickly and effectively through an existing policy
- foster stability and continuity
- maintain the direction of the organisation even during periods of change
- provide the framework for business planning
- assist in assessing performance and establishing accountability
- clarify functions and responsibilities.

Developing and introducing workplace policies

Step 1 – Management Support

It is crucial to have senior management support for the implementation or modification of a policy, especially where policies relate to employee behaviour. The endorsement and modelling of the behaviour by senior managers and supervisors will encourage staff to take the policies seriously. While management support for a policy is an important first step before actively seeking employee feedback on a proposed policy, the idea for the policy and some of its details may in fact come from staff.

Step 2 - Consult with staff

Involve staff in developing and implementing workplace policies to promote stronger awareness, understanding and ownership of the outcome. Staff involvement also helps to determine how and when the policies might apply, and can assist in identifying possible unintentional outcomes of the policy.

Step 3 - Define the terms of the policy

Be explicit. Define key terms used in the policy at the beginning so that employees understand what is meant. The policy should explain what is acceptable and unacceptable behaviour in the workplace. You may wish to include specific examples to illustrate problem areas or unacceptable types of behaviours. For example:

An individual shall be deemed to be under the influence of alcohol if he/she exceeds a blood alcohol level of 0.05% (0.02% for heavy vehicle drivers).

Be clear about who the policy applies to. For example, does it only apply to employees of the company or to contractors and sub-contractors engaged to perform work on business premises? This is particularly important, for example, with occupational health and safety which covers everyone in the workplace.

The policy may also need to contain information about what to do if it is not possible to follow the policy. For example, if you have a policy relating to punctuality, you may need to include a procedure outlining what to do if the employee is going to be late.

The policy should also contain procedures to support the policy in its operation, such as the implications for not complying with the policy.

Example 1: Occupational health and safety

No employee is to commence work, or return to work while under the influence of alcohol or drugs. A breach of this policy is grounds for disciplinary action, up to and including termination of employment.

Example 2: Email policy

Using the organisation's computer resources to seek out, access or send any material of an offensive, obscene or defamatory nature is prohibited and may result in disciplinary action.

Step 4 - Put the policies in writing and publicise them

To be effective, policies need to be publicised and provided to all existing and new employees. This includes casual, part-time and full-time employees and those on maternity leave or career breaks.

Policies should be written in plain English and easily understood by all employees. Consider translating the policies into the appropriate languages for employees whose first language is not English.

Ensure all staff understand what the policies mean. Explain how to comply with the policies and the implications of not complying.

Step 5 - Training and regular referral

The policies may be explained to staff through information and/or training sessions, at staff meetings and during induction sessions for new staff. They should also be reiterated and discussed with staff regularly at staff meetings to ensure they remain relevant.

Copies of policies should be easily accessible. Copies may be kept in folders in a central location or staff areas, in staff manuals and available on the organisation's intranet system.

Step 6 – Implementation

It is important that policies are applied consistently throughout the organisation. A breach of a policy should be dealt with promptly and according to the procedures set out in the policy. The consequence of the breach should also suit the severity of the breach – whether it be a warning, disciplinary action or dismissal.

Step 7 - Evaluate and review

Review policies regularly to ensure they are current and in line with any changes within the organisation. Where policies are significantly changed they should be re-issued to all staff and the changes explained to them to ensure they understand the organisation's new directions. These changes should also be widely publicised.

Policy checklist

A workplace policy should:

- set out the aim of the policy
- explain why the policy was developed
- list who the policy applies to
- set out what is acceptable or unacceptable behaviour
- set out the consequences of not complying with the policy
- provide a date when the policy was developed or updated.

Types of workplace policies

Here are some examples of common workplace policies that could assist your workplace:

- code of conduct
- recruitment policy
- internet and email policy
- mobile phone policy
- non-smoking policy
- drug and alcohol policy
- health and safety policy
- anti-discrimination and harassment policy
- grievance handling policy
- discipline and termination policy
- using social media

Law and Labour regulations

Labour Management Relations

Labour management relations has been defined as ‘the effort to develop harmonious working relations between employees and management in order for a firm to increase productivity, improve quality of work life and remain competitive’.

Objectives of Labour Management Relations

- To enhance a healthy relationship between employers and employees, that is, a sound, harmonious and mutually beneficial relationship.
- To safeguard the interests of employees and management by securing mutual understanding and goodwill between them.
- To avoid industrial and workplace conflict that could affect production and productivity.
- To preventing or reduce labour turnover and frequent absenteeism.

- To nurture and sustain workplace democracy in order to enhance employee participation in decision making and problem solving.
- To eliminate strikes, lockouts and go slows.
- To allow the government some control over private means of production, distribution and service provision in the public interest
- The management should never be caught up in a surprise strike if they have been in good relations and communication with the employees. Those in Management positions should not be members of trade unions.

Other Laws Related to Nursing Practice

The following are some of the laws that are relevant to nursing practitioner

The Public Health Act (cap 242)

This Act is commonly referred to as the mother Act among the Acts of Health Professionals.

It describes the health delivery services in the country.

Pharmacy and Poisons Act (Cap 244)

You should be familiar with this law because of drug prescription and administration.

Medical Practitioners and Dentists Act Cap (253)

In your community health nursing practice or private nurse practice you will be expected to diagnose and treat minor ailments, this encroaches on this law.

Narcotic Drugs and Psychotropic Substances (Control) Act 1994

While dealing with the dangerous drugs, you will be required to be familiar with this law.

This also applies to dealing with drug misuse and abuse.

Mental Health Act Cap (248)

You have been exposed to mental health and psychiatric nursing.

You will interact with such patients in your practice as a general nurse, midwife and community health nurse.

Other Laws Related to Nursing Practice

Medical Laboratory Technicians and Technologists Act (1999)

You will be taking body specimens from patients and clients.

You should therefore be familiar with this law.

Clinical Medicine Act

This law is related to the medical Practitioners and Dentists Act because of diagnosing and treating minor ailments.

Food, Drugs and Chemicals Act

You will be dealing with the nutritional status of the patient. In addition, you are one of the food handlers in the hospital. Familiarise yourself with the law.

Children and Young Persons Act

In your practice you will be interacting with babies, children, adolescents and teenagers. This may be in wards, departments, clinics, maternity, school health and the community. Remember children have rights which you must recognise.

You may be the provider of the health services, advocate or advisor of other health professionals in your multidisciplinary teams. You may have heard of the African Charter on the Rights and Welfare of the Child which was adapted by the 26th ordinary session of the Assembly of Heads of State and Government of the OAU, Addis Ababa, Ethiopia - July 1990.

Leadership skills

Honesty and integrity

When it comes to leadership, honesty is about being transparent and open. You need to be willing to communicate your thoughts and feelings, even if what you're saying is unpopular or uncomfortable. It also means keeping your word as well as following through on any promises you make.

Integrity, on the other hand, goes hand in hand with work ethic and having a good work ethic sets an example. If you always make the effort to make the best possible decisions both in the interest of your company and your staff, employees will look at you as a leader with integrity.

Improve your leadership skills in our immersive course. [Click here to learn more.](#)

Problem solving

A lot of people believe that problem solving is fixing an error irrespective of the cost. In actual fact, many leaders will tell you that a problem could end up being an opportunity, and effective problem solving is about embracing that fact. It's also about avoiding finger-

pointing at all costs; if a problem emerges, it's vital that you don't get caught up trying to blame someone. It's unproductive, wastes time and doesn't solve the issue.

Instead, it's important to look at things in an analytical way. Collecting data is always an effective method when it comes to getting through problems. It avoids having to rely on guesswork and will help you to ensure the problem doesn't happen again. Above all else, it's important that a manager stays positive during a problem. Reacting negatively will impact morale and create panic.

Delegate work to team members' strengths

Recognising your team members' strengths will help create a positive workplace environment and ensure tasks get completed in a timely manner. If possible, carry out either a formal or informal strengths assessment. You can assess the strengths of a team member through observation or even through an interview. Alternatively, you can use a more formal method such as a strength-based questionnaire.

Communicate powerfully and prolifically

Perhaps the most important aspect of being a leader is communication. You can be incredibly innovative and technically gifted but if you lack the skills in order to effectively communicate your ideas and methods, they will struggle to get off the ground.

You can practice your communication skills in realistic VR experiences.

Essentially, powerful communication will help you to influence as well as inspire your team. In order to do that, there are three tools you will need to master:

- Active listening
- Creative questioning
- Responding influentially

Active listening will give you a greater understanding and awareness of any given situation, and the same can be said for creative questioning. Responding in an influential way will help you to convey effective words and messages which will inspire your team.

It can take time, and linking listening, questioning and messaging requires delicate and careful skills. Once you get it right, however, your conversations will be infinitely more constructive and will lead to more successful meetings.

Collaborate and promote teamwork

Promoting teamwork wherever possible will help your team work more cohesively towards the company's vision. A good way to achieve teamwork is through meetings and assigning tasks that require your team to communicate with each other as well as form trust and be able to support each other.

Once the task has been completed, team members will have added respect for each other and value other members' roles. It may also work if you provide incentives or rewards for fulfilling the tasks as this could help to motivate them even more.

Written test

Short response assessment

List all interpersonal skills

Explain the conflict resolution mechanisms

Outline factors that promote teamwork at working station

Reference

Louis A. Allen, Management and Organization, McGraw-Hill Kogakusha, Ltd.

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Learning outcome 4: Control Perioperation Theatre resources

Introduction to the learning outcome

This unit describes the competencies required to control perioperation Theatre resources. This learning outcome involves, Resource management ,Medical legal aspects in Perioperation Theatre ,Creative and Critical thinking,Basic monitoring and evaluation,Observation skills,Interviewing skills and Investigative skills in Theatre.

Performance Standard

1. Objectives for PT activities are set as per work place policy
2. Standard performance in PT is established as per Standard Management Procedures, WHO and work place policy
3. Tools for standard performance are developed as per workplace policy.
4. Performance indicators are identified as per set objectives.
5. Baseline performance is identified and documented as per workplace policy

6. Targeted performance is set and documented as per Workplace policy.
7. Evaluation of performance is carried out as per workplace policy
8. Progress reports are prepared as per workplace policy.

Resource management

Management is the skillful handling or is the skillful handling or treatment of a task. treatment of a task.

Resource management is the skillful handling is the skillful handling of resources.

Resource is anything that can be used to is anything that can be used to assist in the performance of a task

Types of resource:

Human Resource; energy, time, skill, knowledge. , energy, time, skill, knowledge.

Money, Money, used to buy other resources e.g. food equipment used to buy other resources e.g. food equipment

Commodities, Commodities, food, equipment, fabric, wood, fuel, food, equipment, fabric, wood, fuel, electricity, cleaning agent. electricity, cleaning agent.

Community resources Community resources, libraries schools, hospitals.

Legal Aspects in Theatre Nursing

You will start by looking at the term 'legal'. The dictionary defines the word legal as 'required' or 'permitted by law'. Therefore, when we talk of legal aspects in theatre nursing, we are referring to what the law requires us to do in the theatre before, during and after the operation. In your clinical practice as a nurse, you may have participated in nursing a patient who was to undergo an operation. Can you remember what preoperative care was required before the patient could go for the operation.

Preoperative Care

The preoperative care requirements are: You should make sure that the surgeon

explains clearly to the patient what will happen to them.

The surgeon should obtain an informed consent from the patient or parent/guardian/

next of kin for those under age or not in a position to sign (e.g. unconscious person).

The nurse ensures that the patient has signed an informed consent, after the surgeon has explained the advantages and outcomes of the operation.

1. Make sure that the patient observes a 'Nil by oral' rule. The fasting should usually start six hours before the operation.
2. Blood works: All should be within the acceptable ranges e.g. full Haemogram including HB, urea, electrolytes and creatinine.
3. The patient should be counselled and reassured especially those receiving operations such as amputation, or mastectomy.
4. The site to be operated on should be shaved of hair and cleaned with warm soapy water, to reduce the bacteria on the patient's skin. The area shaved should be larger than the incision site.
5. Catheterisation and IV cannula insertion may be necessary depending on the surgery.
6. Observations of vital signs, urine testing for sugars, proteins and acetone should be done.
7. The receiving area nurse should confirm that the above preoperative measures have been taken by the ward nurse in order to allow the patient in theatre.

Some of the preparations form part of the legal requirements before surgery.

In unit one of this module you covered nursing ethics where the importance of confidentiality in nursing practice was stressed. This is another legal requirement. In the definition of legal, the term 'permitted by law' implies that you can only carry out patient care within what the law permits you to do. Therefore, the law gives the patient seeking medical, surgical and nursing care, rights under which they are to be managed.

The dictionary defines rights as any claim that is morally just or legally granted as allowable or due to a person. This brings you to the term 'legal rights of an individual during theatre nursing'.

When it is said that a person has the legal right to informed consent, what is meant is that they must be given information regarding the type of operation to be performed, why it is necessary, and its effects both bad and good, before being requested to sign the consent form for the operation.

However, you must also be aware of the fact that not all patients are given all the information regarding the type of operation they are to undergo. Before you give such information, you need to take factors such as age and level of education into consideration.

No matter how impossible it is to communicate this information to a patient, it would be against the law and their individual rights to ask them to sign the consent form for an operation when they have not fully understood the implications.

It is on this basis that those below the legal age of adulthood (18 years in Kenya) are not legally bound to sign the consent form. It is signed by the parents/guardians on their behalf. In the same way, consent for the mentally ill is sought from their parents/guardians/relatives. It is also important to note that consent for an operation

should be obtained from the patient before they are pre-medicated, as pre-medication drugs have the potential of affecting their reasoning capacity, hence making consent signed not legally binding.

Having looked at confidentiality and informed consent, it is important to mention that the custody and security of the patient before, during and after operation is vested in the theatre team. It has already been implied earlier that by signing the consent form, the patient takes some responsibility for the whole loss of life or part of their body. However, this does not take away the responsibility of the theatre team to ensure the security of the patient's life during the operation.

The legal aspect in theatre nursing involves the care of the patient from the time the patient is accepted in theatre, until they are handed over back to the ward.

For these reasons, the following procedure should be adhered to:

1. Any patient going to theatre must be properly prepared preoperatively.
2. The patient must sign an informed consent, obtained by the surgeon.
3. The patient must be protected from any harm, falls or eventuality, during the stay in theatre.
4. Confidentiality must be observed regarding the patient.
5. Measures must be taken to ensure that the patient taken to theatre is the right one for the intended operation.
6. The items to be used for the operation must be counted and recorded before and after operation to prevent loss of swabs, tubes, blades, forceps, abdominal packs and any instrument used.
7. Theatre nurses must know where the exits are, for use in case of an emergency.
8. Sockets in theatre should be covered during scrubbing to prevent risk of conducting currents. They should also be one meter or more above the floor level.
9. All electrical machines must be checked to ascertain optimum function before use on the patient.

Creative and Critical thinking

Critical thinking is applied by nurses in the process of solving problems of patients and decision-making process with creativity to enhance the effect. It is an essential process for a safe, efficient and skillful nursing intervention. Critical thinking according to Scriven and Paul is the mental active process and subtle perception, analysis, synthesis and evaluation of information collected or derived from observation, experience, reflection, reasoning or the communication leading to conviction for action .

So, nurses must adopt positions that promote critical thinking and refine skills of critical reasoning in order a meaningful assessment of both the previous and the new information and decisions taken daily on hospitalization and use of limited resources, forces you to think and act in cases where there are neither clear answers nor specific procedures and where opposing forces transform decision making in a complex process.

Critical thinking applies to nurses as they have diverse multifaceted knowledge to handle the various situations encountered during their shifts still face constant changes in an environment with constant stress of changing conditions and make important decisions using critical thinking to collect and interpret information that are necessary for making a decision

Critical thinking, combined with creativity, refine the result as nurses can find specific solutions to specific problems with creativity taking place where traditional interventions are not effective. Even with creativity, nurses generate new ideas quickly, get flexible and natural, create original solutions to problems, act independently and with confidence, even under pressure, and demonstrate originality .

The aim of the study is to present the basic skills of critical thinking, to highlight critical thinking as a essential skill for nursing education and a fundamental skill for decision making in nursing practice. Moreover to indicate the positive effect and relation that critical thinking has on professional outcomes.

Go to:

2. CRITICAL THINKING SKILLS

Nurses in their efforts to implement critical thinking should develop some methods as well as cognitive skills required in analysis, problem solving and decision making (5). These skills include critical analysis, introductory and concluding justification, valid conclusion, distinguishing facts and opinions to assess the credibility of sources of information, clarification of concepts, and recognition conditions .

Critical analysis is applied to a set of questions that relate to the event or concept for the determination of important information and ideas and discarding the unnecessary ones. It is, thus, a set of criteria to rationalize an idea where one must know all the questions but to use the appropriate one in this case

The Socratic Method, where the question and the answer are sought, is a technique in which one can investigate below the surface, recognize and examine the condition, look for the consequences, investigate the multiple data views and distinguish between what one knows and what he simply believes. This method should be implemented by nurses at the end of their shifts, when reviewing patient history and progress, planning the nursing plan or discussing the treatment of a patient with colleagues

The Inference and Concluding justification are two other critical thinking skills, where the justification for inductive generalizations formed from a set of data and observations, which when considered together, specific pieces of information constitute a special interpretation

In contrast, the justification is deduced from the general to the specific. According to this, nurse starts from a conceptual framework—for example, the prioritization of needs by Maslow or a context—evident and gives descriptive interpretation of the patient's condition with respect to this framework. So, the nurse who uses drawing needs categorizes information and defines the problem of the patient based on eradication, nutrition or need protection.

In critical thinking, the nurses still distinguish claims based on facts, conclusions, judgments and opinions. The assessment of the reliability of information is an important stage of critical thinking, where the nurse needs to confirm the accuracy of this information by checking other evidence and informants .

Basic monitoring and evaluation

What is a Monitoring and Evaluation Plan?

A monitoring and evaluation (M&E) plan is a document that helps to track and assess the results of the interventions throughout the life of a program. It is a living document that should be referred to and updated on a regular basis. While the specifics of each program's M&E plan will look different, they should all follow the same basic structure and include the same key elements.

An M&E plan will include some documents that may have been created during the program planning process, and some that will need to be created new. For example, elements such as the logic model/logical framework, theory of change, and monitoring indicators may have already been developed with input from key stakeholders and/or the program donor. The M&E plan takes those documents and develops a further plan for their implementation.

Observation skills

Observation Skills: Definition and Examples

When you're applying for jobs, it's imperative that you adequately communicate your strengths and highlight skills that are beneficial for the role you're seeking. Developing your observation skills makes you a more effective communicator and employee, which is why many employers look for candidates that possess this ability. In this article, we explain what observation skills are, explore how you can improve them and utilize them in the workplace and provide ways to highlight these skills during the application process.

What are observation skills?

Observation skills refer to the ability to use all five of your senses to recognize, analyze and recall your surroundings. This practice is often associated with mindfulness because it encourages you to be present and aware of the details of your daily life.

Observation skills are dependent on several other abilities and attributes, such as:

- Communication
- Emotional intelligence
- Critical thinking
- Attention to detail
- Interviewing skills

Investigative skills Control your biases

Good investigators understand what their biases are and work to control them while in investigation mode. Adopt the persona of a sports referee—the neutral arbitrator, without a stake in the outcome. This is easier said than done. You work in organizations and your leadership will have their own set of biases and perhaps even taboo topics. Yet a good investigator must adopt and maintain that open and neutral stance.

2) Take great care in what you label a fact

How to defend your findings and recommendations? Have an impeccable foundation to base your conclusions, rooted in a carefully constructed set of building blocks of evidence, each of which you can demonstrate is true. Avoid single-source information that cannot be corroborated by a second source. Remember the derivation of the word “witness” (to see, hear or know by personal presence and perception). I’m very interested when a witness says “I saw” or “heard” as opposed to “I think.”

3) Improve your interviewing skills

Often the most important evidence is the recollections of witnesses. Being able to gather the information of witnesses is critically important. Only about ten percent of safety professionals tell me they have had formal training in conducting an effective interview. Most learned to interview by watching someone else do it – with no indication that the person they were watching had any formal training or talent in this area. Effective, ethical interviews require both science and art. Science gives you the basics of human behavioral principles that must underpin the interview. The art comes only with practice.

4) Avoid moving prematurely to cause analysis

No root cause analysis tool will fill in gaps in the evidence. Keep the root cause tool and any discussion about causes on hold until you and the investigation team agree you have gathered, organized and assessed all available information and have a firm timeline of the incident. Only then will the root cause methodology be effective in guiding you to identify all appropriate causes.

5) Write more explicit recommendations

Often investigation teams lose their focus and just write something down addressing the causes identified. Use a structured method of crafting your recommendations. Give them

precision and impact. One common structure is the SMART method— recommendations that are Specific, Measurable, Achievable, Realistic and Timely. Rate recent recommendations to see how they measure up to those criteria. I think you will be surprised.

Each of these five areas are totally within your control and do not require any particular support from your organization. Used as a means of self- improvement you'll develop a reputation for the quality of your investigations.

Short answer responses

What are the attributes that enhance proper observation skills

references

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Learning outcome 5: Document PT resources

1.2.2 Introduction to the learning outcome

This unit describes the competencies required to document PT resources. It involves Records keeping in PT and Inventories in Operation Theatre.

1.2.3 Performance Standard

PT resources to be documented are identified in accordance to work place policy

Required recording materials are gathered according to work place policy and SOPs,

Recording is carried out as per workplace policy and SOPs

Inventory is stored as per work place policy and SOPs

Waste is disposed with due regard to environment protection regulations

Inventory Control

Adequate quantities of materials should be available whenever required. Close supervision of movement of materials or consumption rate is a good tool for proper control.

Storage

The materials should be placed in a store within or near the institution. The store should be of adequate size to accommodate all the different materials required, for example, fridges, shelves and cupboards.

The materials should also be stored in appropriate conditions, for example, temperature, light as specified by the manufacturer.

- It is, therefore, important that the nurse manager adopts the proper procedures for materials management

Records keeping in PT

All professional persons need to be accountable for the performance of their duties. — perioperative theatre technician as professionals need to record their work on completion. — A record is a permanent written document that communicates information relevant to a client's health care management. It is a clinical, scientific, administrative and legal document relating to the care given to the individual, family or community.

Purpose of records

Provide data for planning and evaluation

Tools for communication between health workers, the family and other personnel

Indicate plans for the future

Provide baseline data to estimate long-term changes related to patient care

Provide opportunity for evaluation of services

Help in research for improvement of nursing

Types of records

Every organization keeps some records and likewise every department in the hospital has its records such as: 1. Patient's clinical notes 2. Nurses' notes i.e. records of nurses assessment and interventions and care 3. Records of admission and discharge 4. Records of equipment loss and replacement (inventory) 5. Records of personnel performance

Inventories in Operation Theatre

The objective of inventory management is to make appropriate decisions regarding the level of inventory. Any inventory system must specify when an order is to be placed and how many units to order. In practice there are a lot of items involved and it is therefore not practical to model and control each item with equal attention. Classification of inventory is thus necessary. Inventory plays a crucial role in the stock room of the operating theatre and contributes to the loss Garden City is currently experiencing. Investigation of Classification of inventory follows.

Self-assessment

SHORT ANSWER RESPONSES QUESTIONS

1. Explain xy theory of Management
2. State Staffing procedures/recruitment procedures
3. Discuss any three Communication methods
4. Explain any 4 Ethical issues in operating theatre
5. Explain any 10 examples of Interpersonal skills
6. Describe six leadership skills
7. State the 10 effective qualities of a team leader
8. What Is the Importance of Studying Management Theories & Practice

RESPONSES TO SHORT ANSWER RESPONSES

What is X-Y Theory of Management?

Douglas McGregor, an American social psychologist, proposed his famous X-Y theory in his 1960 book 'The Human Side of Enterprise'. Theory X and Theory Y are still referred to commonly in the field of management and motivation, and whilst more recent studies have questioned the rigidity of the model, McGregor's X-Y Theory remains a valid basic principle from which to develop positive management style and techniques. McGregor's XY Theory remains central to organisational development, and to improving organisational culture.

McGregor's X-Y theory is a salutary and simple reminder of the natural rules for managing people, which under the pressure of day-to-day business are all too easily forgotten. Theory X - 'Authoritarian Management' Style

Assumptions made under this theory:

The average person dislikes work and will avoid it if he/she can.

Therefore most people must be forced with the threat of punishment to work towards organisational objectives.

The average person prefers to be directed; to avoid responsibility; is relatively unambitious, and wants security above all else.

His ideas suggest that there are two fundamental approaches to managing people:

Many managers tend toward Theory X and generally get poor results.

Enlightened managers use Theory Y, which produces better performance and results, and allows people to grow and develop.

What is Z-Theory of Management?

Theory Z is often referred to as the 'Japanese' management style, which is essentially what it is. Interestingly, Ouchi chose to name his model 'Theory Z', which apart from anything else tends to give the impression that it's a McGregor idea.

Theory Z essentially advocates a combination of all that's best about Theory Y and modern Japanese management, which places a large amount of freedom and trust with workers, and assumes that workers have a strong loyalty and interest in team-working and the organisation.

Theory Z also places more reliance on the attitude and responsibilities of the workers, whereas McGregor's XY Theory is mainly focused on management and motivation from the manager's and the organisation's perspective.

State Staffing procedures/recruitment procedures

Manpower Planning

Employment of Personnel

Recruitment

Selection

Placement

Training

Training

Development

Promotion and Career Planning

Transfer

Performance Appraisal

Compensation

Determination of Remuneration.

4. Discuss any three Communication methods

Communication Methods

A connected team and a connected organization in today's world is the only way we can achieve greater things. Connections and relations are built with exchange of thoughts and information. Communication serves as a valuable mechanism in building these connections and engagement between people.

Communication is a very important skill and art. There are four different ways we communicate with others. No matter what form of communication we choose, it is important to have the right intention to connect with the other person.

1. Written

It refers to printed or recorded materials such as plans, contracts, memos, minutes of the meeting, requirement analysis document, design documents risk registers and other related written materials. It is used to provide well-thought and well-planned details, thus easily to be reviewed than verbal communication because everything that has been written cannot be altered right away. More so, it can be used for future reference.

Written communication can be time consuming, but is extremely essential for formal scenarios which require approvals and having legal implications as well. Emails have ushered in a revolution in making written communication very fast and thus have become a preferred mechanism for sharing detailed information in fraction of a second.

2. Oral/Verbal

It involves the exchange of message or information by using words verbally through face to face or telephone conversations. It is the most common type of conversation as it paves way to get feedback right after receiving the message. It is known to be spontaneous as well. In verbal communication, the ability to listen and think carefully before speaking is critical.

A person must be careful of the words or expressions to be said. Any word that has been spoken cannot be taken back, so it is important to formulate expressions or sentences carefully.

3. Non-Verbal

Non-verbal communication is sending information without the use of words. It is mostly through one's body language, gestures, eye-contacts, appearance, pitch and tone of one's voice etc. one can communicate huge amount of feelings and emotions.

They say, "It is important to understand what has not been said explicitly".

Non-verbal communication includes, body-language, paralanguage (pitch and tone of voice), appearance etc. Research has shown that nonverbal cues or body language along with facial expressions, tone of voice and body stance account for almost 55% of all the communication that takes place.

4. Visual Communication

"A picture is worth a thousand words". Visual aids such as animation, colour, illustration, graphs & charts, drawing, signs and logos immensely enhance written communication. Visual communication is also used in the right balance for making information sharing more effective and efficient.

5. Explain any 4 ethical issues in operating theatre

You are the chair of your medical center's ethics committee and director of the hospital's ethics consultation service. As an otolaryngologist with a busy surgical practice, you spend a good deal of time in the operating room and understand the special ethical challenges that this patient care center can pose for surgeons and staff. Additionally, you worked recently as the chief of the specialty surgical services.

Your presence is requested for a conference meeting with the hospital's medical director, chief of surgical services, chief hospital nurse, and head nurse of the operating room. During this meeting, these individuals indicate that they are undertaking an effort to review all of the procedural rules and guidelines for the operating room (OR), including the recent statement by the American College of Surgeons on the responsibility of the

primary attending surgeon during surgery. These leaders seek your input and advice regarding ethical considerations that could have a bearing on the revision and/or clarification of the procedural rules and guidelines. They realize that it is important to understand “ethics in the OR” for many of the everyday challenges and dilemmas that arise during the course of performing surgery.

You agree to provide your knowledge and perspectives, and those of other surgeons who participate in the ethics committee’s deliberations, for their consideration. The first set of procedures and rules for the primary attending surgeon that they would like for you to address includes the following:

Informing patients, during the consent process, about staff members who will be a functional part of the surgical team, and alerting them to team members’ roles and experience level; and

Informing patients clearly and concisely about the process of overlapping, sequenced, and multidisciplinary surgeries under the direction of the primary attending surgeon, and about how this practice might affect their care

6. Explain any 10 examples of Interpersonal skills

Active listening.

Teamwork.

Responsibility.

Dependability.

Leadership.

Motivation.

Flexibility.

Patience

7. Describe six leadership skills

The Six Styles of Leadership

Visionary — mobilize people toward a vision. ...

Coaching — develop people for the future. ...

Affiliative — create emotional bonds and harmony. ...

Democratic — build consensus through participation. ...

Pacesetter — expect excellence and self-direction. ...

Commanding — demand immediate compliance

8. State the 10 effective qualities of a team leader

The qualities of an effective team leader inspire the trust and respect of the team and stimulate production within the workplace.

A Clear Communicator. ...

Strong Organization Skills. ...

Confident in the Team. ...

Respectful to Others. ...

Fair and Kind. ...

An Example of Integrity. ...

Influential in Core Areas. ...

Willing to Delegate

9. What Is the Importance of Studying Management Theories & Practice

People who create management theories rely upon observation and mathematics in order to construct a model for business activities. Management practice relies upon case studies and the individual experiences of managers when dealing with workplace situations. Since both schools of management have flaws and benefits, a business owner should study both styles of management in order to improve profitability.

Benefits of Learning

Employees most commonly leave their jobs due to poor management practices, a situation that increases costs and lowers the talent present in a business. Business owners should understand good management practices through personal research or formal education in order to create a business model that can improve employee productivity, eliminate redundancy in processes and increase retention rates.

Limitations of Theory and Practice

Management theories face limitations, because models of human behavior in a business do not consider all of the variables that can impact profitability. Different businesses face different issues with employees, financial resources and the use of technology.

For example, a workplace of single mothers requires a company to focus more on family leave, a consideration that a theorist might not work into a general business model. Management practice can also result in flawed management behavior, because managers cannot see the business as a whole and instead rely solely upon their own experience.

Creating Uniformity of Management

Management theories work best from a macro perspective, such as when a business determines the appropriate model for management as a whole or starts a large project that it has never attempted before. The formal structure of this type of management works best for large corporations that have a top-down management structure which requires uniformity in order to accomplish goals, even if this model slightly decreases productivity.

Personalized Management Style

Since management practices rely upon the opinions of managers and employees or a case study in a particular area of business, they work best for informal organizations. Little doubt exists as to whether a management model will work, because the track record of the model speaks for itself. Management practices focus more on dynamics between groups, which allows managers more flexibility in making decisions and helps employees function together as a unit when they work together on a project.

Putting Theory Into Application

Business owners should mix management practices and theories together based upon their business model. A small business owner will usually study and implement management practices, because he requires flexibility to sustain his business mode. As his business grows, he may add some elements of management theory in order to formalize the decision making and leadership qualities of management.

